



Prehospital emergency care

Pocket reference

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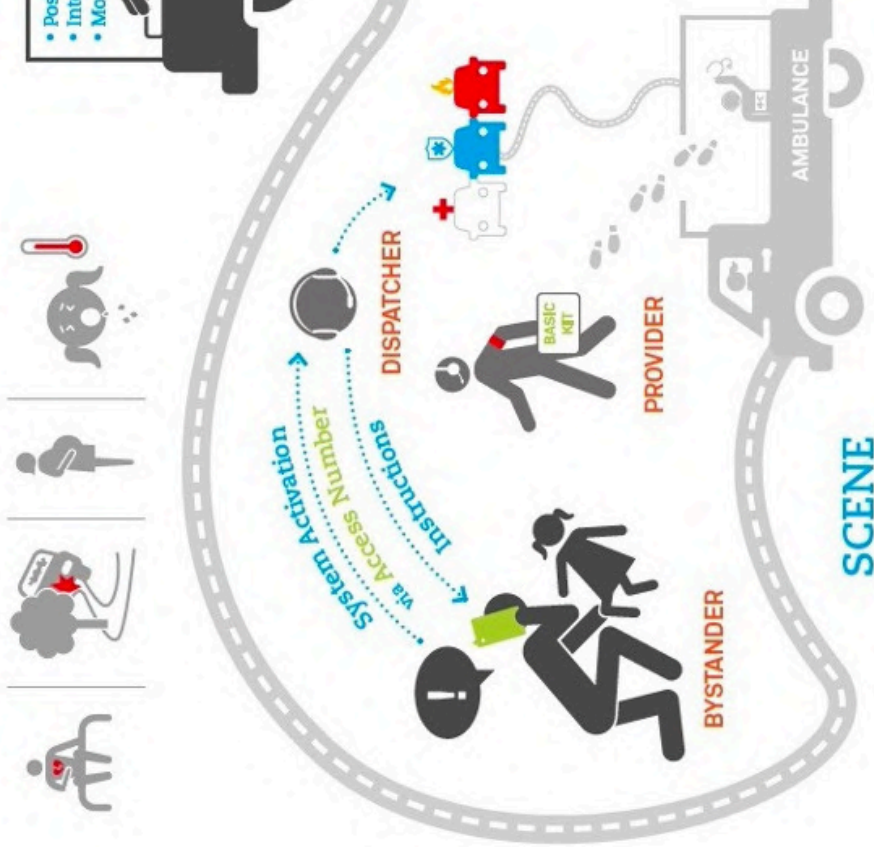
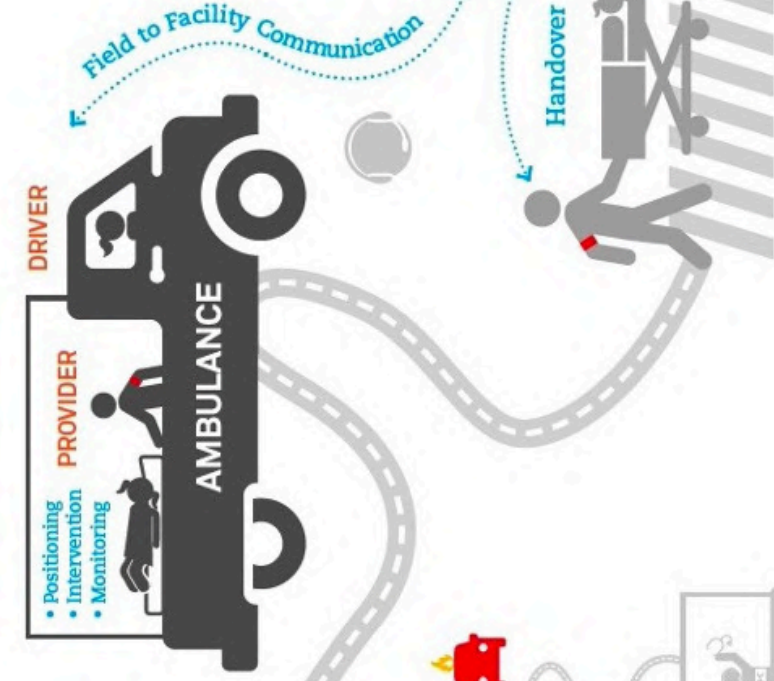
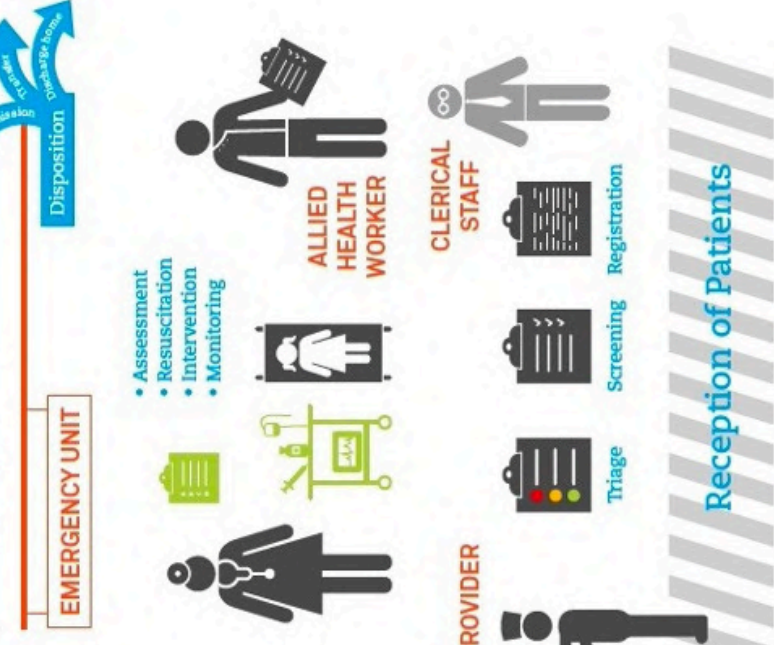
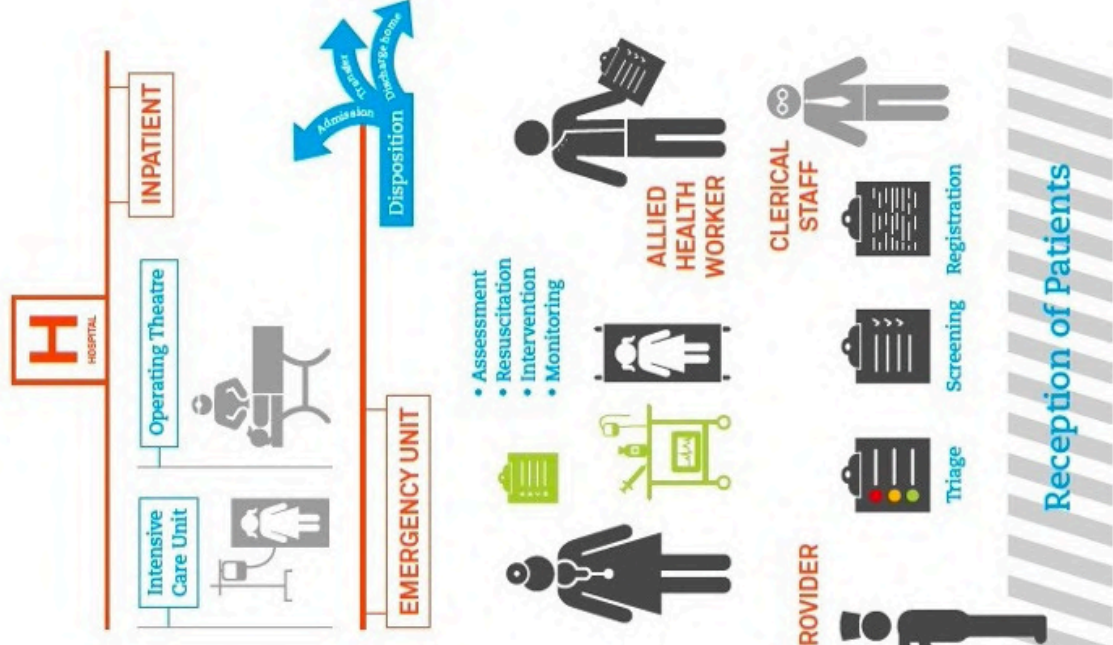
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EMERGENCY CARE SYSTEM FRAMEWORK

All around the world, acutely ill and injured people seek care every day. Frontline providers manage children and adults with injuries and infections, heart attacks and strokes, asthma and acute complications of pregnancy. An integrated approach to early recognition and management reduces the impact of all of these conditions. Emergency care could address over half of the deaths in low- and middle-income countries.

HUMAN RESOURCES **FUNCTIONS** **EQUIPMENT, SUPPLIES, INFORMATION TECHNOLOGIES**



FACILITY

TRANSPORT

RECEPTION OF PATIENTS

STANDARD APPROACH TO ALL PATIENTS

***Oxygen administration - difficulty in breathing**

- Mild to moderate: nasal cannula 2-6 Lpm
- Severe: non-rebreather mask 10-15 Lpm
- If not improving, BVM assisted respirations with 15 Lpm
- Be guided by O₂ sats (where available) - maintain > 92%

!IV fluid resuscitation in non traumatic emergencies

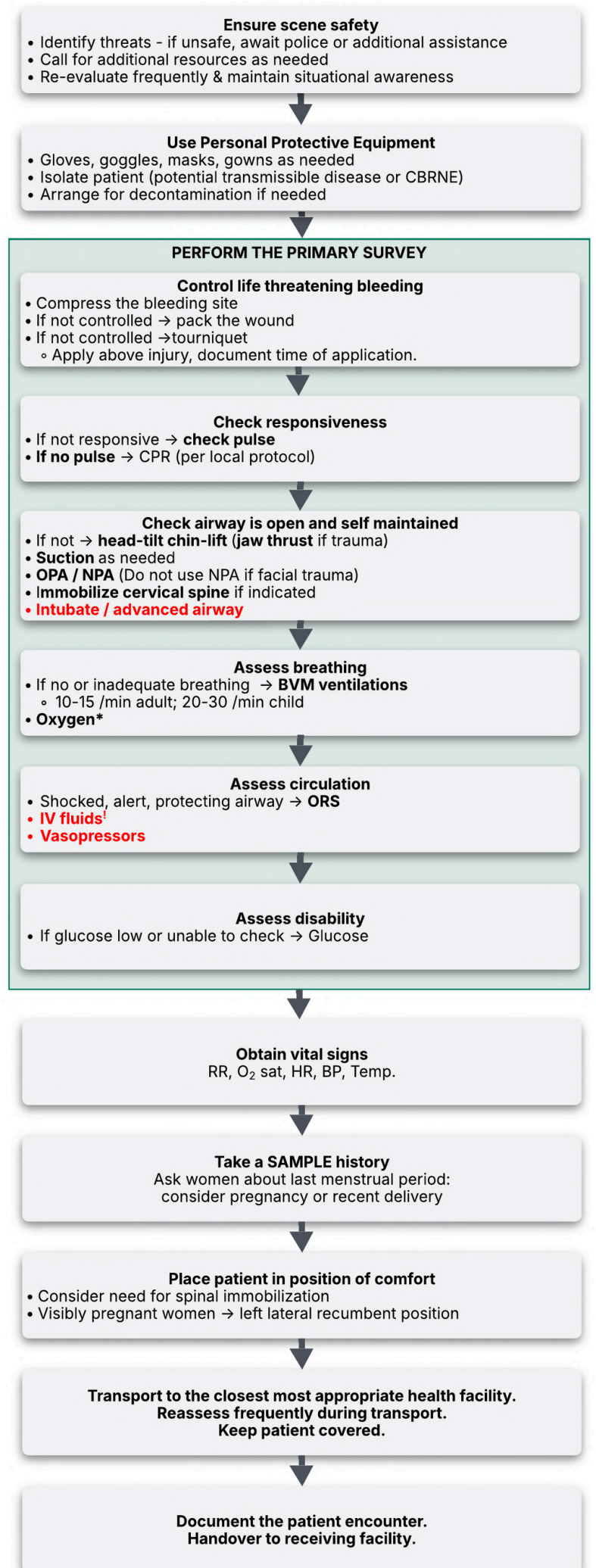
- If shocked → IV fluids
 - Adults: 250-500 mL bolus over 5 minutes
 - Children: 20 mL/kg over 10 minutes
 - Malnourished children: 10 mL/kg over 10 minutes
- If signs of shock remain
 - repeat IV bolus
 - IV vasopressors if unresponsive to second bolus
- If improving → slow IV fluids titrated by vital signs and clinical condition

Glucose administration

- Buccal - smear 25g of oral glucose on gums or inside cheek
- IV - 20 mL of 50% glucose
- Recheck glucose after 5 minutes. Repeat if no improvement

Follow relevant clinical protocol for condition specific management

Be alert for signs of abuse or human trafficking - contact appropriate authorities (per local protocol)



NORMAL ADULT VITAL SIGNS

Pulse rate: 60–100 beats per minute

Respiratory rate: 10–20 breaths per minute

Systolic blood pressure >90 mmHg

Oxygen Saturation > 92%

Estimating systolic blood pressure

(not reliable in children and the elderly):

Carotid (neck) pulse → SBP ≥ 60 mmHg

Femoral (groin) pulse → SBP ≥ 70 mmHg

Radial (wrist) pulse → SBP ≥ 80 mmHg

SAMPLE History

Signs & Symptoms

Allergies

Medications

PMH

Last oral intake

Events

SPECIAL CONSIDERATIONS IN THE ASSESSMENT OF CHILDREN



- Children have bigger heads and tongues, and shorter, softer necks than adults. Position airway as appropriate for age.
- Always consider foreign bodies.



- Look for signs of increased work of breathing (e.g. chest indrawing, retractions, nasal flaring).
- Listen for abnormal breath sounds (e.g. grunting, stridor, or silent chest).

AGE	NORMAL RESPIRATORY RATE (breaths per minute)
<2 months	40–60
2–12 months	25–50
1–5 years	20–40



- Signs of poor perfusion in children include: slow capillary refill, decreased urine output, lethargy, sunken fontanelle, poor skin pinch.
- Look for signs of anaemia and malnourishment (adjust fluids).
- Remember that children may not always report trauma and may have serious internal injury with few external signs.

AGE (in years)	NORMAL HEART RATE (beats per minute)
<1	100–160
1–3	90–150
4–5	80–140



- Always check AVPU.
- Hypoglycaemia is common in ill children.
- Check for tone and response to stimulus.
- Look for lethargy or irritability.



INFANTS AND CHILDREN HAVE DIFFICULTY MAINTAINING TEMPERATURE

- Remove wet clothing and dry skin thoroughly. Place infants skin-to-skin when possible.
- For hypothermia, cover the head (but be sure mouth and nose are clear).
- For hyperthermia, unbundle tightly wrapped babies.

DANGER SIGNS IN CHILDREN

- Signs of airway obstruction (unable to swallow saliva/drooling or stridor)
- Increased breathing effort (fast breathing, nasal flaring, grunting, chest indrawing or retractions)
- Cyanosis (blue colour of the skin, especially at the lips and fingertips)
- Altered mental status (including lethargy or unusual sleepiness, confusion, disorientation)
- Moves only when stimulated or no movement at all (AVPU other than "A")
- Not feeding well, cannot drink or breastfeed or vomiting everything
- Seizures/convulsions
- Low body temperature (hypothermia)

ESTIMATED WEIGHT in KILOGRAMS for CHILDREN 1–10 YEARS OLD:

$$[\text{age in years} + 4] \times 2$$

Prehospital medications: basic ambulance provider

MEDICATION	DOSAGE	INDICATION
Acetylsalicylic acid (Aspirin)	Oral tablet: 300 mg 300 mg (preferably chewed or in water) immediately as single dose.	Concern for acute coronary syndrome
Activated charcoal	Powder or liquid suspension. Adults: 50-100 g (1 g/kg) PO Children: 1 g/kg PO Avoid if ingested household products, paraffin, petrol, corrosive poisons (acids/bases), iron, lithium, or alcohol.	Poisoning
Adrenaline (Epinephrine)	Solution: 1 mg in 1 mL ampoule (1:1000) Adults: 50 kg or above: 0.5 mg IM (0.5 mL of 1:1000) 40 kg: 0.4 mg IM (0.4 mL of 1:1000) 30 kg: 0.3 mg IM (0.3 mL of 1:1000) Repeat every 5 minutes as needed. Children: Anaphylaxis: 0.15 mg IM (0.15 mL of 1:1000). Repeat every 5 minutes as needed. Severe Asthma: 0.01 mg/kg IM up to 0.3 mg. Repeat every 15 minutes as needed.	Anaphylaxis/severe allergic reaction and severe wheezing
Carbetocin	Solution: 100 mcg in 1 mL Give a single dose of 100 mcg IM following a vaginal delivery.	Prevention of postpartum haemorrhage
Diazepam	Rectal solution: 2 mg per mL Adults: First dose: 10 mg PR Second dose after 10 minutes: 10 mg PR Children: First dose: 0.5 mg/kg PR Can repeat half of first dose after 10 minutes if seizures/convulsions continue. MONITOR BREATHING CLOSELY in all patients given diazepam.	Seizures/convulsions
Glucose (Dextrose)	Solution: 50% dextrose (D50) 2–5 mL of 50% dextrose OR sugar solution in buccal space.	Hypoglycaemia
Magnesium sulfate	Solution: 1 g in 2 mL ampoule, 5 g in 10 mL ampoule (50% or 500 mg/mL) 10 g IM (5 g in each buttock). If transport delays continue: Give 5 g of 50% solution IM in alternate buttocks every 4 hours.	Eclampsia or pregnant with seizure/convulsion
Metoclopramide	Oral tablet: 10 mg (hydrochloride); oral liquid: 5 mg/5 mL Adults: 10 mg PO stat Children and adolescents: 0.1-0.2 mg/kg PO ; max 10 mg/dose.	Nausea/ vomiting

MEDICATION	DOSAGE	INDICATION
Misoprostol PO	Oral tablet: 200 µg Prevention: 600 µg PO stat Treatment: 800 µg SL stat.	Prevention and treatment of postpartum haemorrhage
Ondansetron	Oral solid: 4 mg, 8 mg; oral liquid: 4 mg/5 mL Adults: 4-8 mg PO stat Children: PO <15 kg: 0.2 mg/kg/dose; 15 to 30 kg: 4 mg; >30 kg: 8 mg Not to be given in children 1 month or younger.	Nausea/ vomiting
Oxytocin	Solution: 10 IU in 1 mL ampoule Immediately after birth / identifying haemorrhage: give 10 IU IM If placenta manually removed / uterus does not contract: repeat 10 IU IM.	Prevention and treatment of postpartum haemorrhage
Paracetamol (acetaminophen)	Oral tablet: 250 mg, 500 mg; rectal suppositories: 250 mg, 500 mg Adults: 500 mg–1 g PO/PR every 6 hours Max 4 g daily or max 2 g daily if liver impairment/ cirrhosis Children: 10–15 mg/kg PO/PR up to four times per day.	Mild to moderate pain, fever, headache
Salbutamol (Albuterol)	Inhaler: 100 mcg per puff Adults: Prime with 5 puffs and give 2 puffs via spacer every 2 minutes until improved. Children: Prime with 5 puffs and give 2 puffs into spacer. Keep spacer in mouth for 3–5 breaths. Repeat until 6 puffs given for < 5 years, or 12 puffs for > 5 years. For severe wheezing, above doses can be given several times in an hour.	Severe wheezing

Prehospital medications: advanced ambulance provider

MEDICATION	DOSAGE	INDICATION
Acetylsalicylic acid (Aspirin)	Oral tablet: 300 mg 300 mg (preferably chewed or in water) immediately as single dose.	Concern for acute coronary syndrome
Activated charcoal	Powder or liquid suspension. Adults: 50-100 g (1 g/kg) PO Children: 1 g/kg PO Avoid if ingested household products, paraffin, petrol, corrosive poisons (acids/bases), iron, lithium, or alcohol.	Poisoning
Adrenaline (Epinephrine)	Solution: 1 mg in 1 mL ampoule (1:1000) Adults: 50 kg or above: 0.5 mg IM (0.5 mL of 1:1000) 40 kg: 0.4 mg IM (0.4 mL of 1:1000) 30 kg: 0.3 mg IM (0.3 mL of 1:1000) Repeat every 5 minutes as needed Children: Anaphylaxis: 0.15 mg IM (0.15 mL of 1:1000). Repeat every 5 minutes as needed Severe Asthma: 0.01 mg/kg IM up to 0.3 mg. Repeat every 15 minutes as needed	Anaphylaxis/severe allergic reaction and severe wheezing
	Infusion (adults): Give continuous IV infusion of 0.01 to 0.5 µg/kg/minute; titrate based on target BP or evidence of end-organ perfusion.	Distributive / Cardiogenic shock
Atropine	Solution: 1 mg (sulfate) in 1 mL ampoule Adults: 2 mg IV , every 3-5 minutes, doubling until clinical effect achieved. Children: 0.05 mg/kg IV , every 3-5 minutes, doubling until clinical effect achieved. Not to be given in children 3 months or younger.	Poisoning
Carbetocin	Solution: 100 mcg in 1 mL Give a single dose of 100 mcg IM/IV following a vaginal delivery.	Prevention of postpartum haemorrhage
Diazepam	Rectal solution: 2 mg per mL solution 5 mg/ 1 mL ampoule Adults: First dose: 10 mg slow IV push or 20 mg PR Second dose after 10 minutes: 5 mg slow IV push or 10 mg PR Children: First dose: 0.2 mg/kg slow IV push or 0.5 mg/kg PR . Can repeat half of first dose after 10 minutes if seizures/convulsions continue MONITOR BREATHING CLOSELY in all patients given diazepam.	Seizures/convulsions
Ergometrine	Solution: 200 µg (hydrogen maleate) in 1 mL ampoule Give 0.2 mg IV ; repeat every 2-4 hours as needed, max 5 total doses.	Prevention and treatment of postpartum haemorrhage

MEDICATION	DOSAGE	INDICATION
Glucose (Dextrose)	Solution: 50% dextrose (D50), 25% dextrose (D25), or 10% dextrose (D10) Adults and children greater than 40 kg: 25–50 mL IV of D50, or 125–250 mL IV of D10 Children up to 40 kg: 5 mL/kg IV of D10 (preferred) 2 mL/kg IV of D25 1 mL/kg IV of D50 If no IV access: 2–5 mL of 50% dextrose OR sugar solution in buccal space.	Hypoglycaemia
Glyceryl trinitrate	Sublingual tablet: 500 mcg 500 mcg SL every 5 minutes up to 3 doses if SBP > 100 mmHg.	Concern for acute coronary syndrome
Isosorbide dinitrate	Sublingual tablet: 5 mg 5–10 mg SL every 2 to 4 hours if SBP > 100 mmHg.	Concern for acute coronary syndrome
Magnesium sulfate	Solution: 1 g in 2 mL ampoule, 5 g in 10 mL ampoule (50% or 500 mg/mL) Give 4 g IV slowly over 20 minutes (Take 8 mL (four 2 mL vials) and dilute with 12 mL of sterile water. This gives 20 mL of 20% solution or 4 g of magnesium sulfate. Administer as an IV loading dose over 5 min.) AND give 10 g IM (use the 5 g/10 mL solution) with 1 mL of 2% lidocaine in each buttock. If unable to give IV, give 10 g IM injection only (as above, 5 g in each buttock). If seizures/convulsions recur after 15 minutes give additional 2 g (10 mL of 20% solution) IV over 5 minutes. If transport delays continue: Give 5 g of 50% solution IM (with 1 mL of 2% lidocaine) in alternate buttocks every 4 hours.	Eclampsia or pregnant with seizure/convulsion
Metoclopramide	Oral tablet: 10 mg (hydrochloride); oral liquid: 5 mg/5 mL; solution: 5 mg/mL (hydrochloride) in 2 mL ampoule. Adults: 10–20 mg IV stat; 10 mg PO stat Children and adolescents: 0.1–0.2 mg/kg PO; max 10 mg/dose.	Nausea/ vomiting
Midazolam	Solution: 1 mg/mL Adults: 0.5–2.5 mg IV over 1 to 2 minutes; repeat every 2 to 5 minutes as needed; titrate to clinical effect; usual total dose: 5 mg Children: 0.1–0.15 mg/kg IM, max 10 mg; 0.25–0.5 mg/kg PO, max 20 mg IV dose: • ≥6 months to <6 years: 0.05–0.1 mg/kg; max dose: 6 mg. • ≥6 years: 0.025–0.05 mg/kg; max dose: 10 mg. • Adolescents: 1–2.5 mg over ≥2 minutes; max dose: 10 mg.	Sedation
	Adults: 10 mg IV repeat after 5 minutes if still having seizure. Children: 0.2 mg/kg IV repeat after 5 minutes if still having seizure.	Seizure
Misoprostol PO	Oral tablet: 200 µg Prevention: 600 µg PO stat Treatment: 800 µg SL stat.	Prevention and treatment of postpartum haemorrhage

MEDICATION	DOSAGE	INDICATION
Morphine	Solution: 10 mg in 1 mL ampoule IV stat doses of 0.05 mg to 0.1 mg/kg/dose titrated (typically 3-7 mg per dose). Repeat as needed Use with caution in the elderly, children and obese patients. MONITOR BREATHING CLOSELY in all patients given morphine. Monitor respiratory rate, sedation, nausea and vomiting.	Severe pain
Naloxone	Solution: 400 µg/mL (hydrochloride) in 1 mL ampoule IV: 100 µg single dose OR IM: 400 µg single dose May repeat every 5 minutes as needed. May require 0.4 mg/hr infusion for several hours for long-acting opioids.	Opioid overdose
Ondansetron	Oral solid: 4 mg, 8 mg; oral liquid: 4 mg/5 mL; solution: 2 mg/mL in 2 mL ampoule. Adults: 4-8 mg PO/IM/IV stat Children: IV 0.15 mg/kg/dose IV stat; max 8 mg/dose PO <15 kg: 0.2 mg/kg/dose; 15 to 30 kg: 4 mg; >30 kg: 8 mg Not to be given in children 1 month or younger.	Nausea/ vomiting
Oxytocin	Solution: 10 IU in 1 mL ampoule Immediately after birth / identifying haemorrhage: Give 10 IU IM/IV AND start IV fluids with 20 IU/L at 60 drops/minute. Once placenta is delivered, continue IV fluids with 20 IU/L at 40 drops/minute if still bleeding. If placenta manually removed / uterus does not contract: repeat 10 IU IM . Continue IV fluids with 20 IU/L at 20 drops/minute for 1 hour after bleeding stops. Max Dose: 3 L of IV fluids containing oxytocin.	Prevention and treatment of postpartum haemorrhage
Paracetamol (acetaminophen)	Oral tablet: 250 mg, 500 mg; rectal suppositories: 250 mg, 500 mg Adults: 500 mg–1 g PO/PR every 6 hours Max 4 g daily or max 2 g daily if liver impairment/ cirrhosis Children: 10–15 mg/kg PO/PR up to four times per day.	Mild to moderate pain, fever, headache
Salbutamol (Albuterol)	Inhaler: 100 mcg per puff Adults: Prime with 5 puffs and give 2 puffs via spacer every 2 minutes until improved. Children: Prime with 5 puffs and give 2 puffs into spacer. Keep spacer in mouth for 3–5 breaths. Repeat until 6 puffs given for < 5 years, or 12 puffs for > 5 years. Nebulizer: (Adults) 5 mg in 5 mL sterile saline. (Children) 2.5 mg in 3 mL sterile saline. For severe wheezing, above doses can be given several times in an hour.	Severe wheezing
Tranexamic acid	Solution: 100 mg/mL in 10 mL ampoule. Adult: 1 g in 200 mL IV over 10 minutes (within 3 hours of injury) Follow initial dose with a continuous infusion of 1 g over 8 hours. 1 g IV as soon as possible after birth. Repeat after 30 minutes if bleeding continues, or if bleeding restarts within 24 hours of first dose.	Severe bleeding following injury Treatment of postpartum haemorrhage

General instructions for surface cleaning and disinfecting in patient care areas

Step 1.

Gather supplies

- Clean set of personal protective equipment (PPE) for each area cleaned.
- Bucket containing detergent (soapy water).
- Bucket containing disinfectant solution.



- At least 2 clean cloths for each area for cleaning (e.g. a single patient space).
 - More cloths may be needed if space is heavily contaminated.



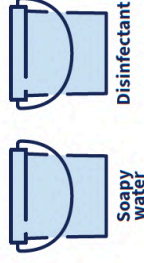
Step 3.

- For general cleaning, wear a gown (or apron) and gloves.

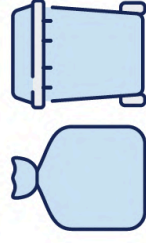


Step 2.

- Perform hand hygiene.



- Waste bin or bag.

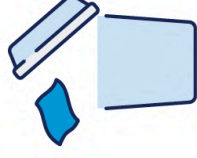


Step 4.

- Clean surfaces using a cloth soaked in soapy water.



- After cleaning, dispose of the cloth appropriately as waste or laundry.



Step 5.

- Disinfect high-touch surfaces (or all surfaces if isolation area) using a cloth soaked in disinfectant.



- Allow surfaces to remain untouched and unused until after the contact time specified by the disinfectant manufacturer

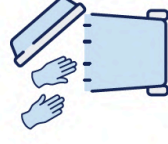


- After completing the disinfecting step, dispose of the cloth appropriately as waste (if single use) or place in laundry.



Step 7.

- Perform hand hygiene.



How to fold and use a cloth for cleaning and disinfection

Step 1.

- Use two clean cloths at minimum (one for cleaning, one for disinfection) for each patient space.



Step 2.

- Fold cloth in half.



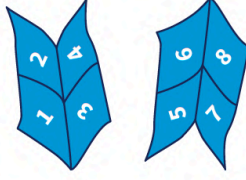
Step 3.

- Then fold the cloth in half again.



Step 4.

- You now have eight different surfaces on the cloth (4 surfaces on front, 4 surfaces on back).



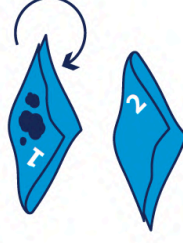
Step 5.

- Soak the cloth in each bucket only once. Do not dip the cloth in the bucket a second time as this will contaminate the solution.



Step 6.

- Switch to a different cloth surface when soiled or when cleaning a different object or furnishing in the patient space to avoid spreading contamination.



Step 7.

- When all sides have been used, dispose of the cloth appropriately as waste (if single-use) or place in laundry container and use a new cloth to continue the task. Always use clean cloths when moving to a different patient's space.

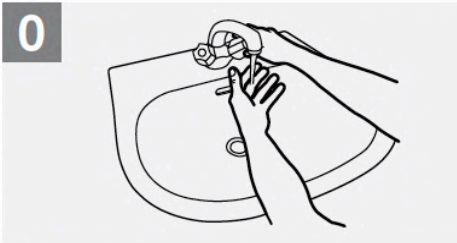


How to Handwash?

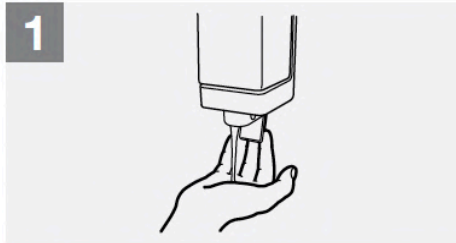
WASH HANDS WHEN VISIBLY SOILED! OTHERWISE, USE HANDRUB



Duration of the entire procedure: 40-60 seconds



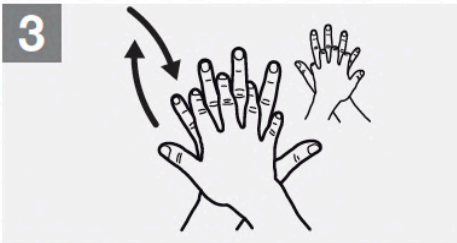
0 Wet hands with water;



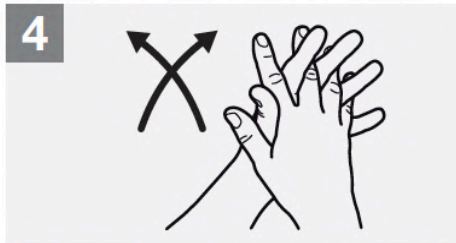
1 Apply enough soap to cover all hand surfaces;



2 Rub hands palm to palm;



3 Right palm over left dorsum with interlaced fingers and vice versa;



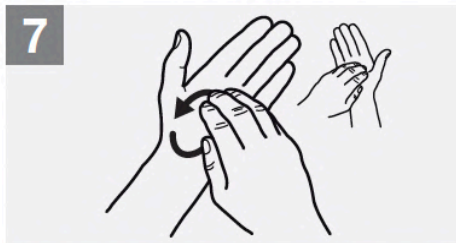
4 Palm to palm with fingers interlaced;



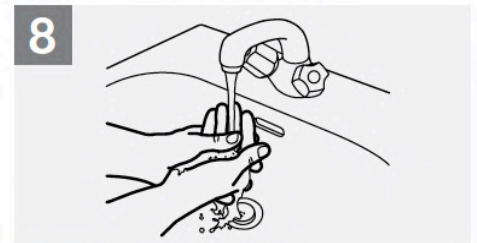
5 Backs of fingers to opposing palms with fingers interlocked;



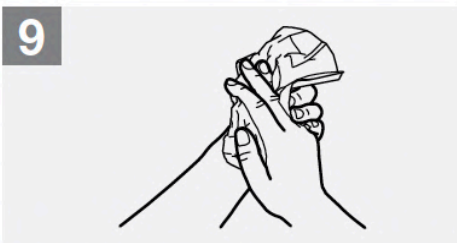
6 Rotational rubbing of left thumb clasped in right palm and vice versa;



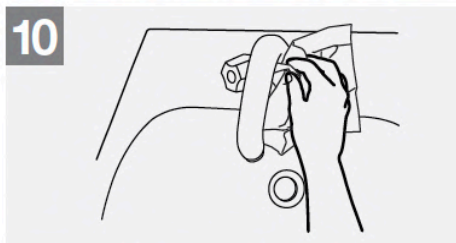
7 Rotational rubbing, backwards and forwards with clasped fingers of right hand in left palm and vice versa;



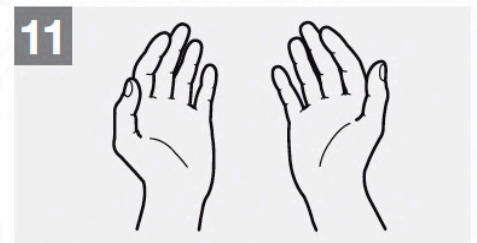
8 Rinse hands with water;



9 Dry hands thoroughly with a single use towel;



10 Use towel to turn off faucet;



11 Your hands are now safe.



World Health Organization

Patient Safety

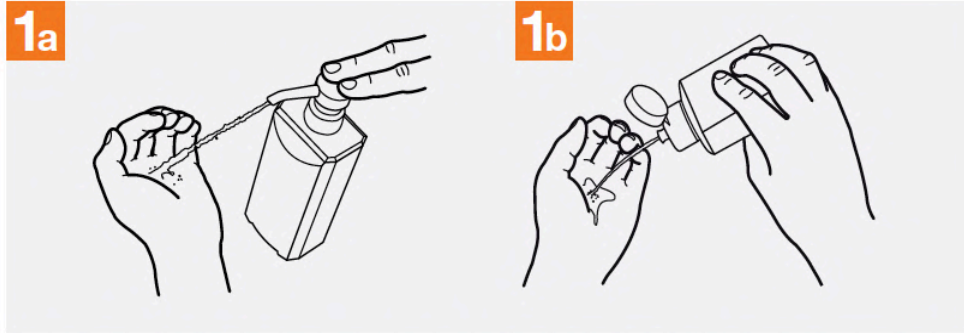
A World Alliance for Safer Health Care

SAVE LIVES
Clean Your Hands

How to Handrub?

RUB HANDS FOR HAND HYGIENE! WASH HANDS WHEN VISIBLY SOILED

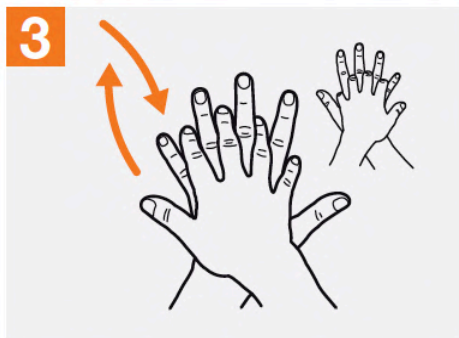
 **Duration of the entire procedure: 20-30 seconds**



Apply a palmful of the product in a cupped hand, covering all surfaces;



Rub hands palm to palm;



Right palm over left dorsum with interlaced fingers and vice versa;



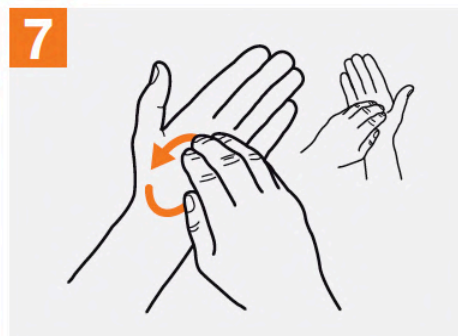
Palm to palm with fingers interlaced;



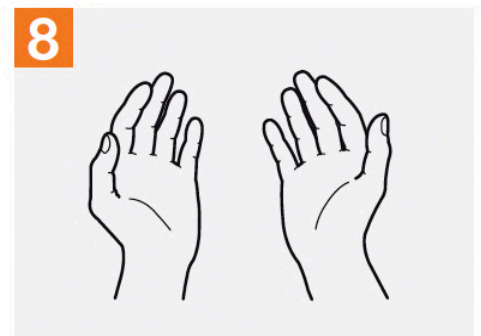
Backs of fingers to opposing palms with fingers interlocked;



Rotational rubbing of left thumb clasped in right palm and vice versa;



Rotational rubbing, backwards and forwards with clasped fingers of right hand in left palm and vice versa;



Once dry, your hands are safe.



**World Health
Organization**

Patient Safety

A World Alliance for Safer Health Care

SAVE LIVES
Clean Your Hands

Facility pre-arrival report form

Use this tool to help facilitate efficient and safe communication during handover of a patient from prehospital provider (community first aid responder or ambulance provider) to higher level of care.

KEY CONSIDERATIONS IN NOTIFYING A RECEIVING FACILITY:

Always confirm the name of the contacted receiving facility via closed loop communication method (e.g. have the receiving facility confirm they are receiving the transmission).	☐
If there are multiple receiving facilities in the system, have the receiving facility identify itself during the communication.	☐
Keep reports as brief as possible and use clear speech.	☐



ALWAYS REPORT THE FOLLOWING INFORMATION TO THE RECEIVING FACILITY:

Confirm that the receiving facility can clearly hear the transmission.	☐
Prehospital provider ID number or identifier.	☐
State the reason for contacting the receiving facility or physician (e.g., consult, notification of transport).	☐
Transport priority and estimated time of arrival.	☐
Number of patients.	☐
Report patient's: age, gender, chief complaint, and mechanism of injury or onset of illness.	☐
Level of consciousness.	☐
Complete set of pertinent vital signs that could be obtained.	☐
Pertinent positive, and negative, physical findings.	☐
Pertinent medical history that may relate to, or complicate, the chief complaint.	☐
Interventions already initiated, patient's response (including pertinent vital signs trends), and any problems encountered (e.g., unable to control airway or stop haemorrhage).	☐
Other special situations (e.g., infectious disease, contaminated patient, hazardous material exposure requiring decontamination prior to entry of receiving facility).	☐
High-acuity conditions requiring additional support include trauma with shock, cardiac arrest, stroke, neonate, or obstetrics delivery imminent	☐



ALWAYS CONFIRM THAT THE REPORT HAS BEEN RECEIVED IN FULL:

Always confirm via closed loop communication method that the receiving facility has correctly noted all of the transmitted information	☐
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Interfacility Acute Transfer Checklist

For use by transport team*

Patient Name:				Date of Transfer:			
Modality of transfer:	Ground (ambulance)	Air	Sea	Time of departure:		Time of arrival:	
Sending facility:				Sending provider:	Name Signature		
Receiving facility:				Receiving provider:	Name Signature		
Reason for transfer:				Primary diagnosis/syndrome:			

1. Information gathering - Review: ☐ Patient identification and receiving facility/unit contact. ☐ Current or recent diagnostic & imaging results. ☐ Recent clinical course & note changes in physiological status that may need to be addressed prior to departure or during transfer. ☐ Current vital signs, medications, & clinical condition.				and ensure that: ☐ Urgent clinical interventions are delivered before transfer. ☐ Handover from treating clinician has occurred. ☐ Current treatment plan and prescription chart are reviewed. ☐ Family/caretaker is informed of transfer & destination. ☐ There is a plan to manage possible complications.							
2. Transfer preparation - Determine: ☐ Equipment requirements & pack appropriately including delivery and newborn packs if required. ☐ Oxygen requirements for duration of transfer, including preparation for delays. ☐ Optimal care provider/team for patient needs (e.g. neonates). ☐ Safety or security concerns and review with medical control. ☐ Contact modality during transport (phone, radio etc.). ☐ Considerations for heat or cold stress and plan for addressing				and ensure that: ☐ Equipment is charged & spare batteries available. ☐ Special circumstances are considered (e.g., infectious diseases or non-standard equipment requirements such as neonatal warmer). ☐ Vehicle has been checked & refuelled. ☐ Required medications are available and have not expired. ☐ The receiving facility has agreed to receive the patient. ☐ Equipment & medication can withstand temperature fluctuations.							
3. Patient preparation~ - Check that: ☐ Airway is unobstructed or secured with device if necessary. ☐ Breathing is adequate or ventilation is optimised. ☐ If intubated, endotracheal tube position is confirmed, documented, and secured. ☐ If ventilated, patient is trialled on transport ventilator & settings optimized for patient condition. ☐ Circulatory status (blood pressure, heart rate, perfusion) is adequate or is being actively managed. ☐ All bleeding has been controlled, and wounds covered. If post-partum bleeding, ensure TXA and uterotonics given. ☐ Ensure antibiotics are given for all patients with sepsis or who meet clinical criteria.				☐ Any fractures, including spinal restriction, are stabilised. ☐ Pain and sedation are adequately addressed. ☐ Neurologic status is assessed and blood glucose > 4 mmol/l (70mg/dL). ☐ All required indwelling attachments (IV access, catheters, drains) have adequate capacity and are correctly placed, & secured. ☐ Patient is attached to transport monitoring equipment. ☐ All needed interventions that cannot be done safely during transport have been performed at the sending facility. ☐ Patient is packaged to ensure access to critical assessment locations during transport and minimise pressure area injury. ☐ Patient is stable enough to withstand transport stress. If not, make alternative plan with care team.							
PAUSE: Before proceeding with transfer, double check all conditions are met for safe patient transport!											
4. Before departure - Check that: ☐ Patient is secured to the stretcher and stretcher is secured. ☐ Patient is covered to maintain normal body temperature. If newborn patient, maintain ongoing kangaroo care. ☐ All referral notes & relevant clinical documentation for handover are present. ☐ All transport equipment is functioning properly. ☐ All equipment is secured or stowed.								☐ Adequate amount of oxygen is present for the duration of the transfer, including preparation for delays. ☐ All infusions have been drawn for the entire journey, as well as spare supplies. ☐ Adequate amounts of medication are in stock and present for the duration of transfer, including preparation for delays. ☐ Personal items and medications issued to patient are present.			
During transport – Ensure that: ☐ Patient monitoring is continuous. ☐ Any changes in airway, breathing, circulation, disability or exposure (ABCDE) status are recognised and interventions immediately begun. ☐ Oxygen is transferred to mobile source/transport vehicle main supply.								☐ Electronic equipment is plugged into ambulance power supply. ☐ All emergency equipment is in reach. ☐ Receiving facility is updated on estimated time of arrival. ☐ Providers and equipment are secured in event of crash. ☐ Documentation is completed. ☐ Family members accompany the patient if safe and appropriate.			
6. On arrival - Ensure that: ☐ Report provided to the receiving team using Situation-Background-Assessment-Recommendation (SBAR) structure. ☐ Referral documentation is provided to the receiving team. ☐ Patient is transitioned onto facility equipment.								☐ Transport vehicle is cleaned and restocked. ☐ Sending facility & dispatch is informed on completion of transfer. ☐ Transfer team has received any significant feedback from the receiving facility.			

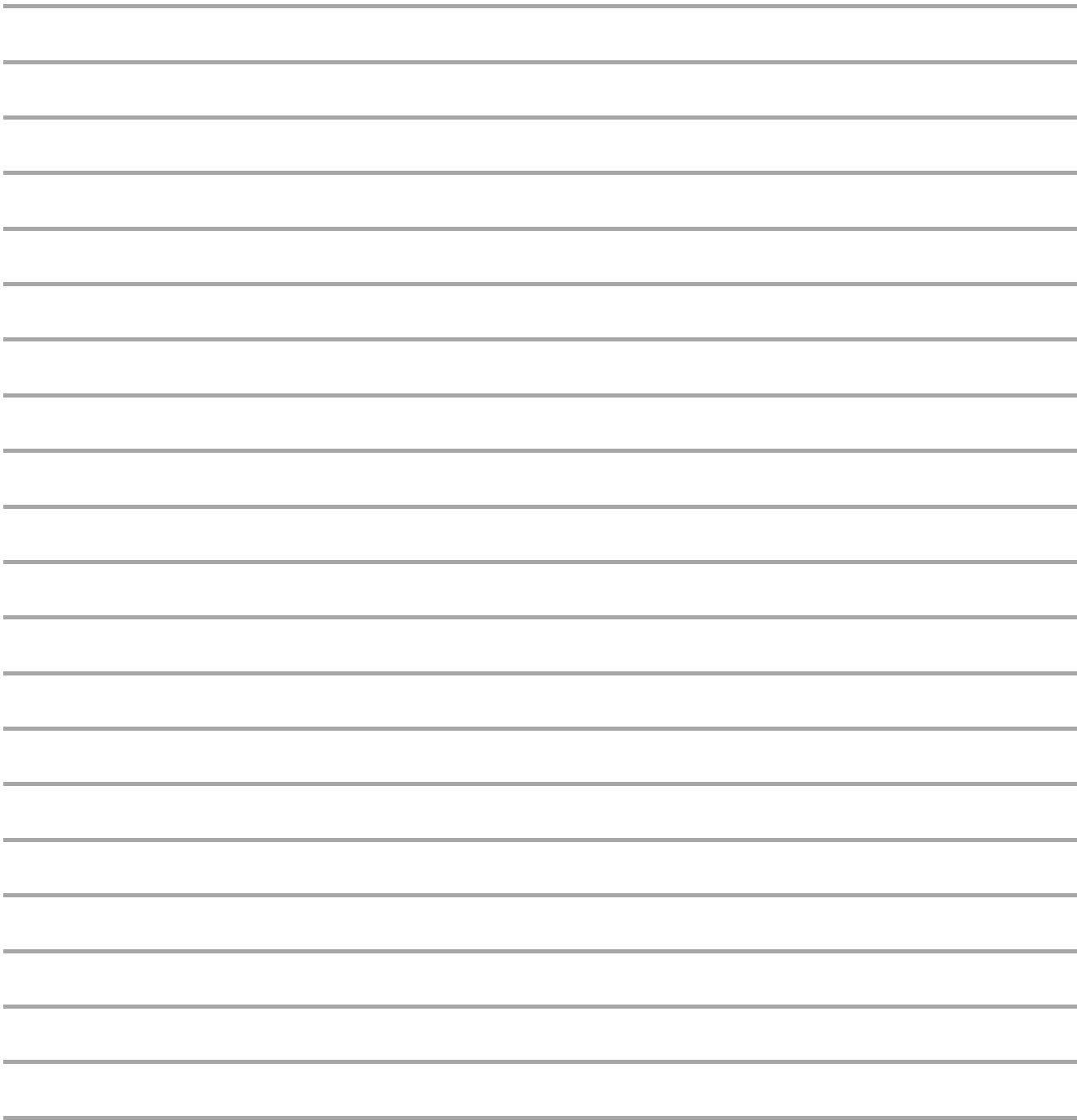
* Interfacility transfer requires a driver AND a provider. If a provider is not available, seriously consider the risk/benefit of transport to the patient. ~
As per WHO-ICRC Basic Emergency Care: <https://www.who.int/publications/i/item/9789241513081>

SBAR Handover Tool - out of hospital

Use this tool to help facilitate efficient and safe communication during handover of an emergency patient from community, clinic or prehospital ambulance transfer.



S Situation	Identify yourself, your role & location ☐ Identify patient (name, age, sex) ☐ State major problem (medical or trauma) ☐ State reason for transfer or handover ☐ (e.g. needs ongoing emergency care for bleeding)
B Background	Describe: What the patient is complaining of ☐ When the patient became ill or injured ☐ Where the patient became ill or injured ☐ How the patient became ill or injured ☐ Any past medical or surgical history, medications or allergies ☐
A Assessment	Describe any findings and care provided for: Check for major bleeding ☐ Airway problem ☐ Breathing problem ☐ Circulation problem ☐ Disability problem ☐ Exposure ☐ Describe any medication taken or given ☐
R Recommendation	State your recommendation (what you feel should happen next) ☐ (e.g. transfer for ongoing emergency care) Describe any concerns ☐ (e.g. social, security, infectious risk)
Confirmation: Ask the receiver to repeat back key information and clarify any questions. 	



PREHOSPITAL FLOW

